

#EndTB Webinar

WHO POLICY BRIEF ON TB AND PRIMARY HEALTH CARE: SYNERGIES AND OPPORTUNITIES TOWARDS UNIVERSAL HEALTH COVERAGE



The primary health care approach

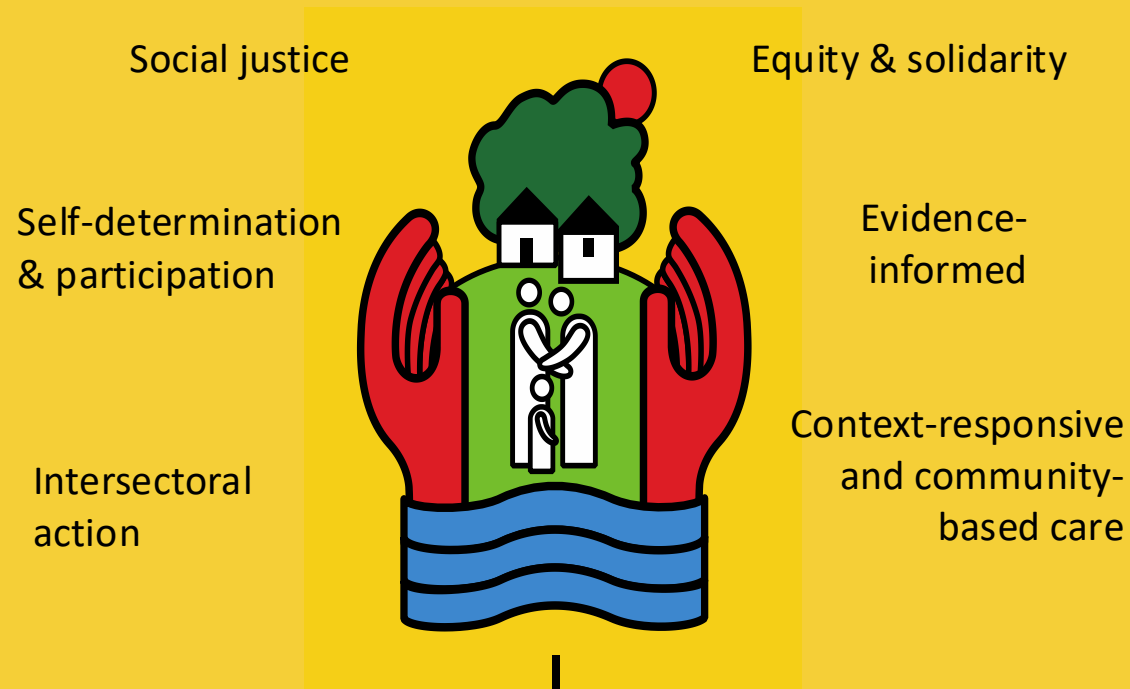
Foundations, history, concepts and impact on performance

Faraz Khalid

18 August 2025

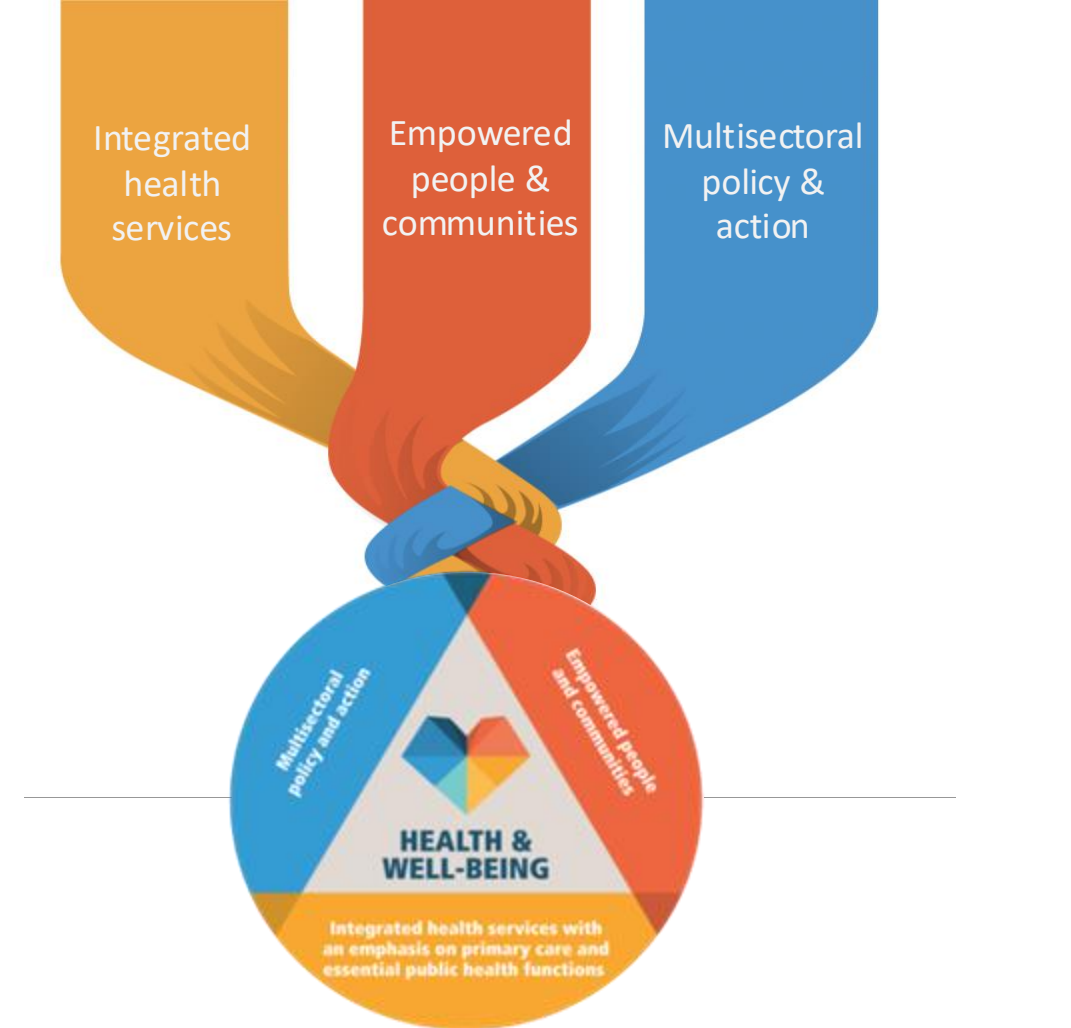
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What is a *PHC* approach?



1978

Declaration of Alma-Ata
Normative principles



2018

Declaration of Astana
Core components

“Primary health care is one of the **most important** concepts in public health –
– and also one of the **most misunderstood.**”

Dr Tedros Adhanom Ghebreyesus
WHO Director-General

Remarks at the Investing in the radical reorientation of
health systems towards Primary Health Care UNGA side event
21 September 2023

Global Reports & Declarations



Declaration of
Alma-Ata

1978

2008

2010

2016

2018

2020

2022

2024

World Health
Report: PHC
Now More
Than Ever

World Health
Report: Health
Systems
Financing:
The Path
to Universal
Coverage

WHO
Framework
on integrated
people-centered
health services



Vision for
PHC in the 21st
century



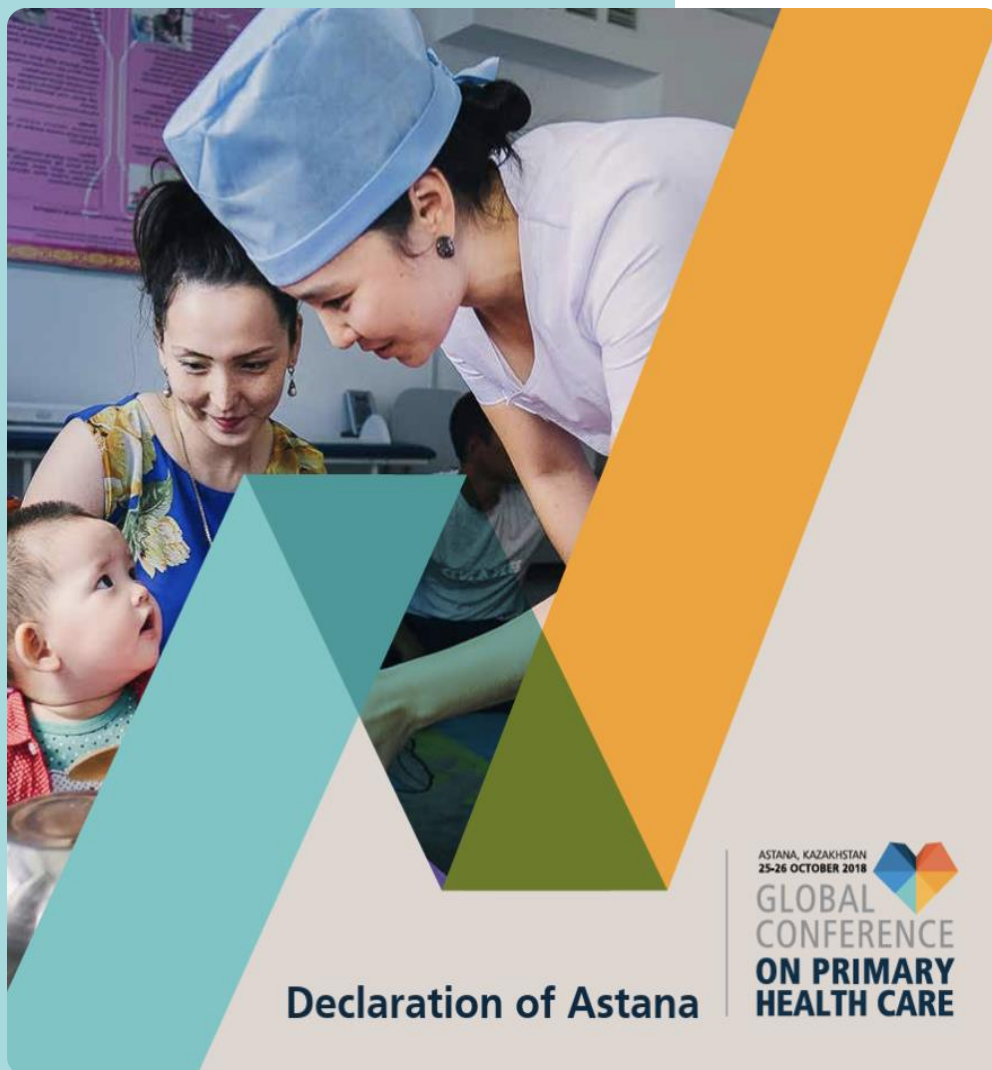
WHO/UNICEF
Operational
framework
for PHC



WHO/UNICEF
Measurement
framework
for PHC



Implementing
the PHC
approach:
a primer



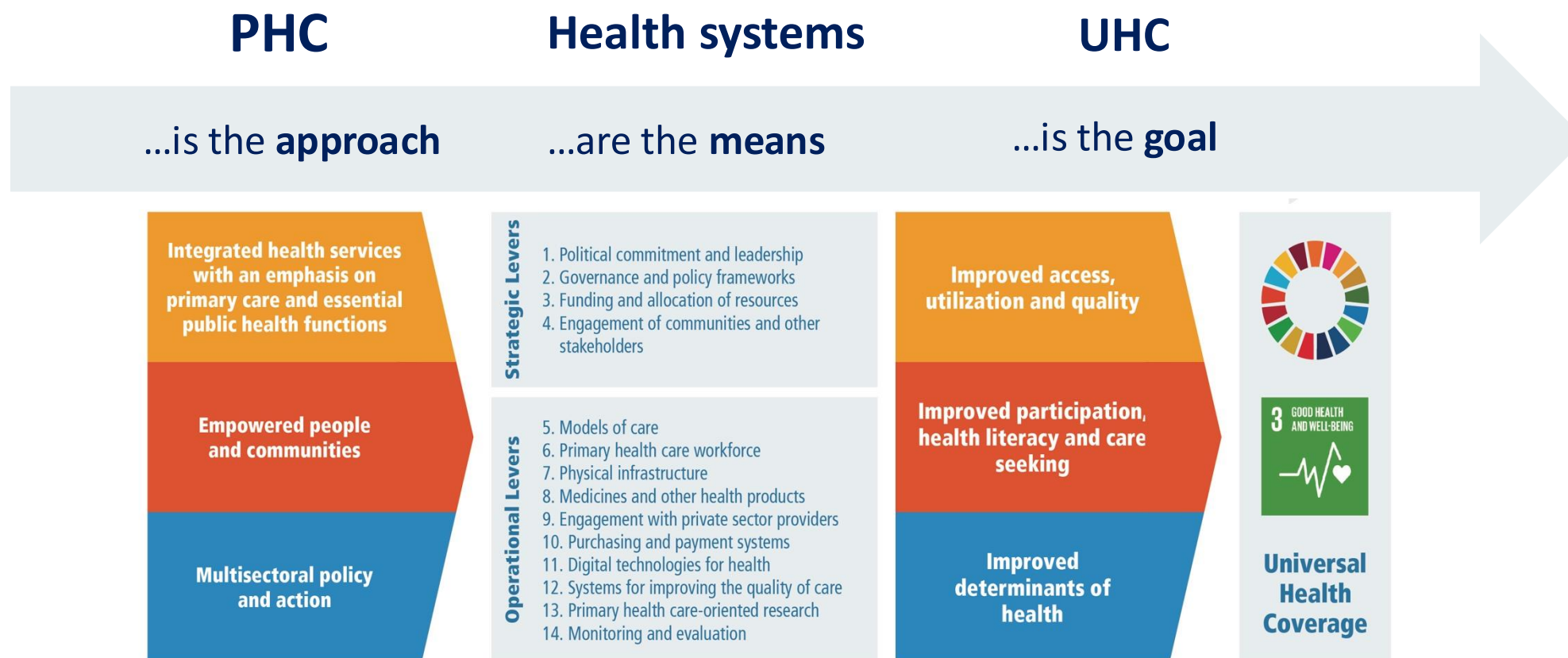
The 2018 Declaration of Astana renewed a commitment to PHC that captured four decades of vision, principles and experience.

The declaration defined PHC as:

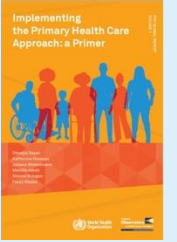
“ a whole of society approach to health that aims equitably to maximize the level and distribution of health and well-being by focusing on people’s needs and preferences (both as individuals and as communities), as early as possible along the continuum of health promotion and disease prevention to treatment, rehabilitation and palliative care, and as close as feasible to people’s everyday environment. ”
(WHO, 2018)



Operationalizing “PHC-oriented health systems for UHC”



Why a renewed global focus on PHC?



1

Multiple rapid
social changes



2

Changing and emerging
health challenges



3

Common conceptual
& semantic pitfalls



4

Building on 40 years
of evidence for PHC



1. Multiple rapid social changes

Complex global trends that impact health at the individual and population levels:

- Uneven economic growth and growing inequity
- Technological advances
- Forced migration
- Urbanization
- Changing family structures and the evolving role of women
- Climate change
- Consumer movement
- The changing roles and rights of women worldwide
- Pandemic threats
- Armed conflicts
- **An evolving geo-political landscape and shrinking development assistance for health**



2. Changing and emerging health challenges

New patterns of health and illness

- From **acute** and **episodic** issues to **complex** and **chronic** conditions
- From **single** issues to **multiple concurrent** issues
- From discrete preventive **interventions** to health-promoting **environments**
- From **medical** issues to **medico-social** issues
- From **epidemics** to **pandemics** and new **emerging global threats**
- From an emphasis on **sanitation** to consideration of **environmental and climate-related** threats.

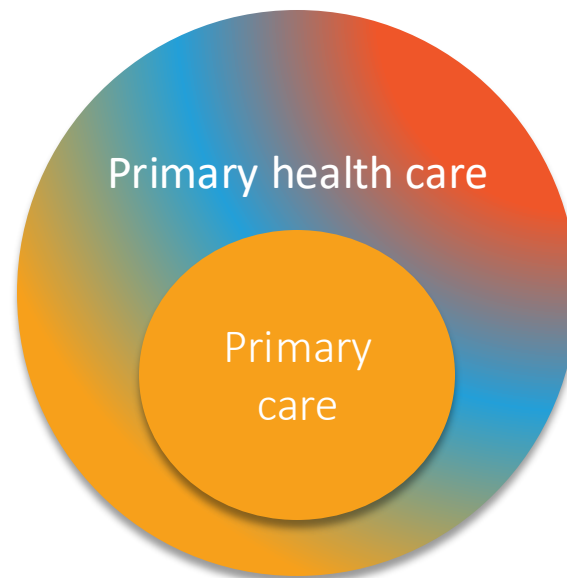
3. Common conceptual & semantic pitfalls

Primary health care

Primary health care is a **system-wide and whole-of-society approach**.

It includes “primary care” at the core of **integrated health services** but extends beyond the delivery of services.

is **NOT**



Primary care

Primary care is **part of** the service-related component of PHC and together with **essential public health function**, forms the **core** of all integrated health services.

A key process in the health system that supports first-contact, **accessible, continued, comprehensive** and **coordinated person-centred care**.

Improved health outcomes, including for mental and child health, and NCDs



Improved services (better quality, access, continuity, comprehensiveness, coordination)



Improved efficiency, reducing unnecessary use of (costly) specialists and hospitals



4. The evidence base for adopting a PHC-orientation



High user satisfaction and better self-reported health



Reduced financial hardship, narrowed outcome gaps and improved equity

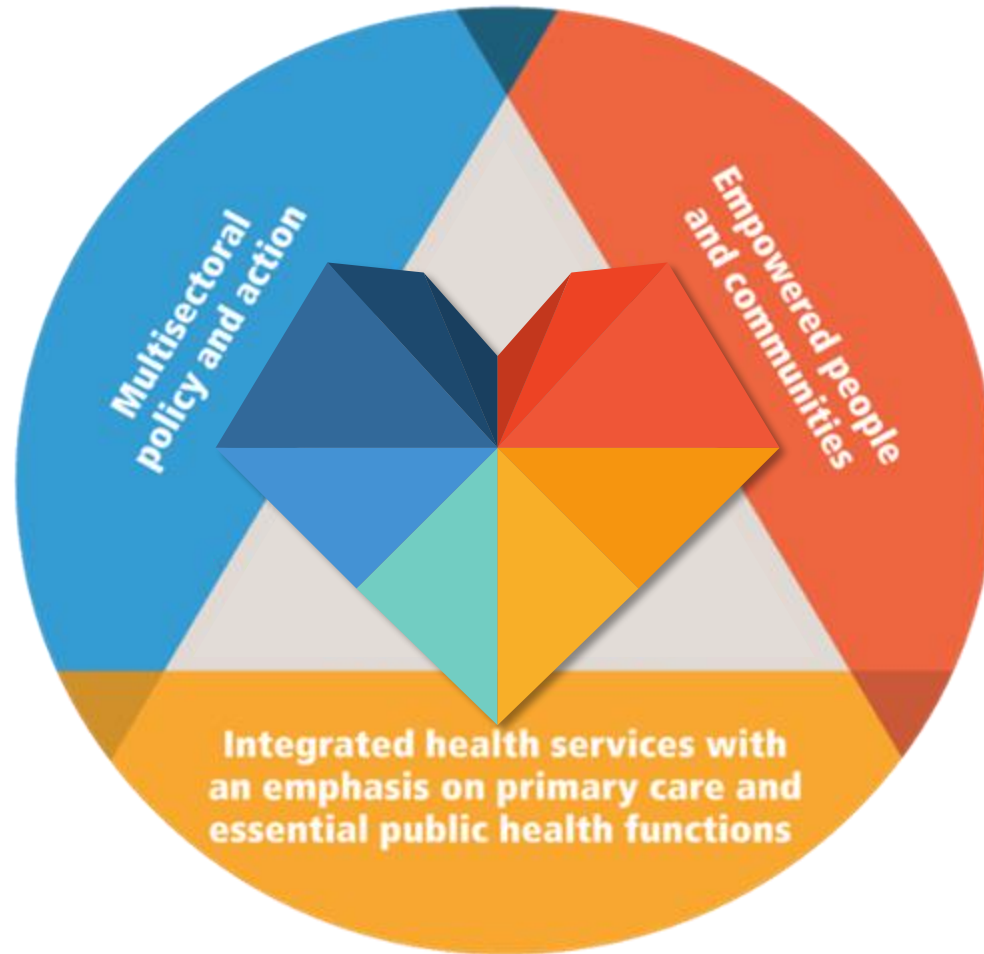


Reinforces emergency preparedness and resilience



TB and PHC

An approach to ensure the needs of people and communities are systematically and holistically addressed...



Individual care that is organized around the needs of people; integrated across services/sectors and public health functions that are informed by a defined population (population-based management)

People are recognized as active participants in their health, and supported to make informed decisions; mechanisms are in place to enable communities

Policies are in place to identify those most in need (marginalized populations) and to work across sectors to address the determinants of health

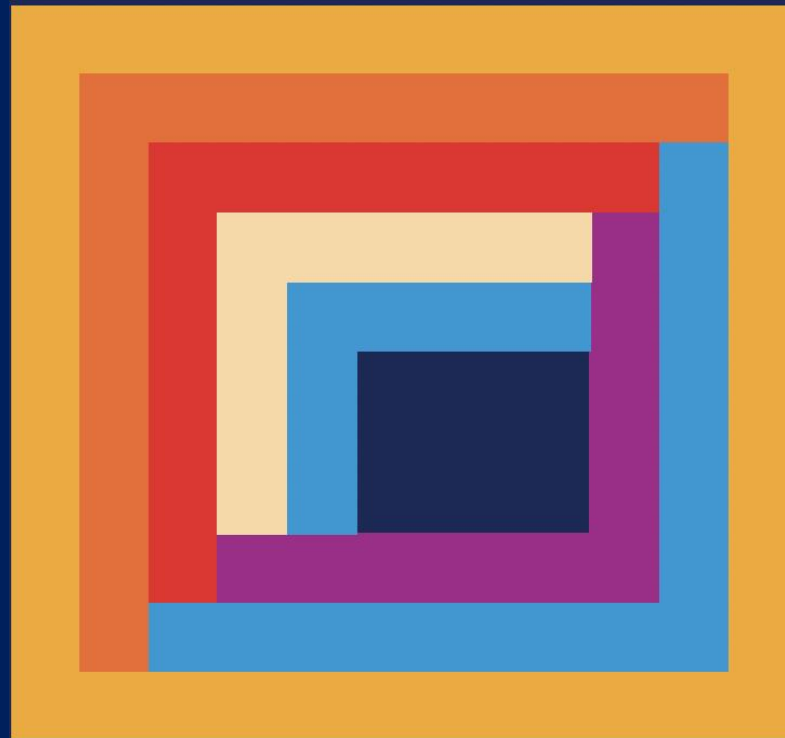
Thank you!

The primary health care approach
Foundations, history, concepts and
impact on performance

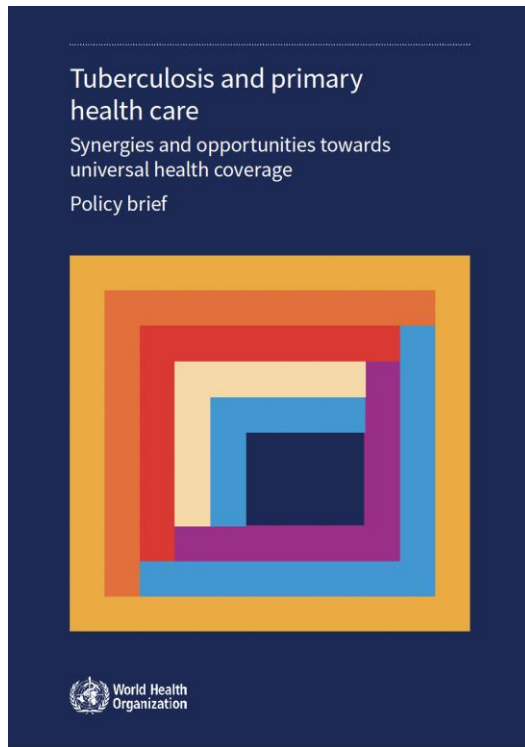
Tuberculosis and primary health care

Synergies and opportunities towards universal health coverage

Policy brief



About the policy brief



1

Why this brief matters?

Background and scope



2

PHC + TB: A powerful alliance

Synergies between the PHC approach and the TB response



3

What helps and what holds us back

Enablers of and barriers to synergistic actions for the PHC approach and ending TB



4

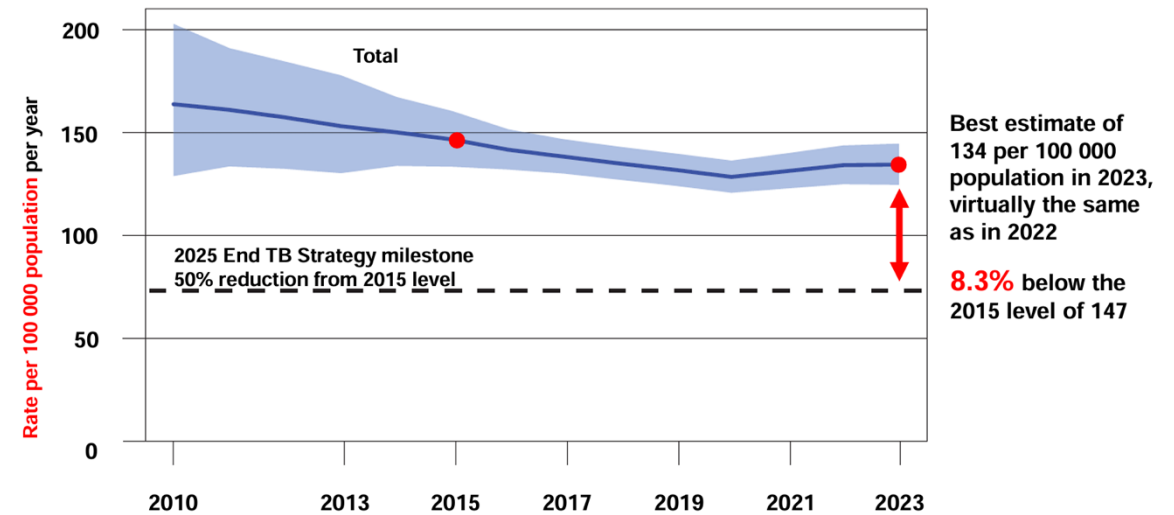
The path forward

Advancing towards PHC-oriented health systems for ending TB

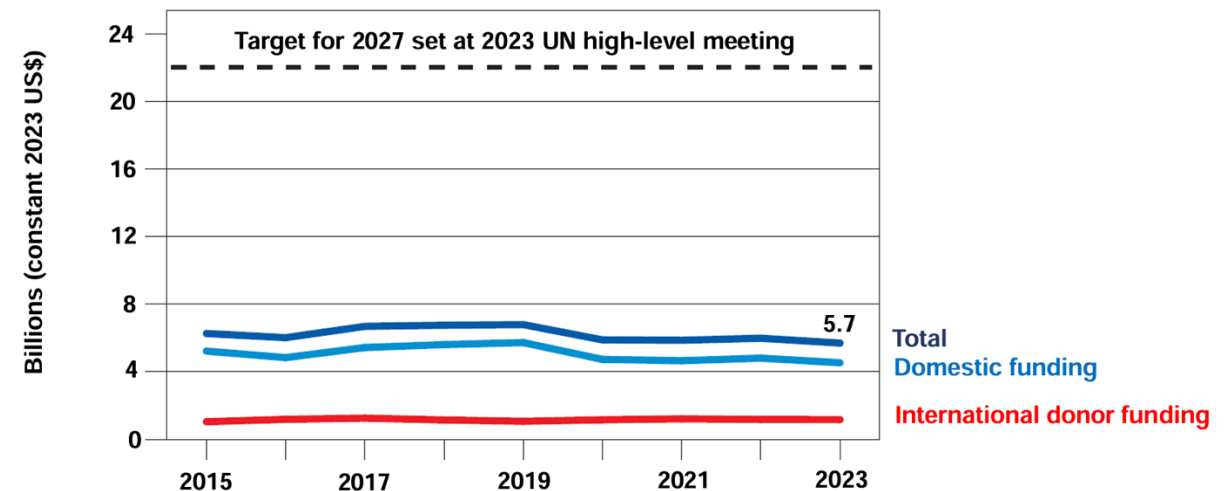


TB and PHC: why does this matter? Global tuberculosis challenge

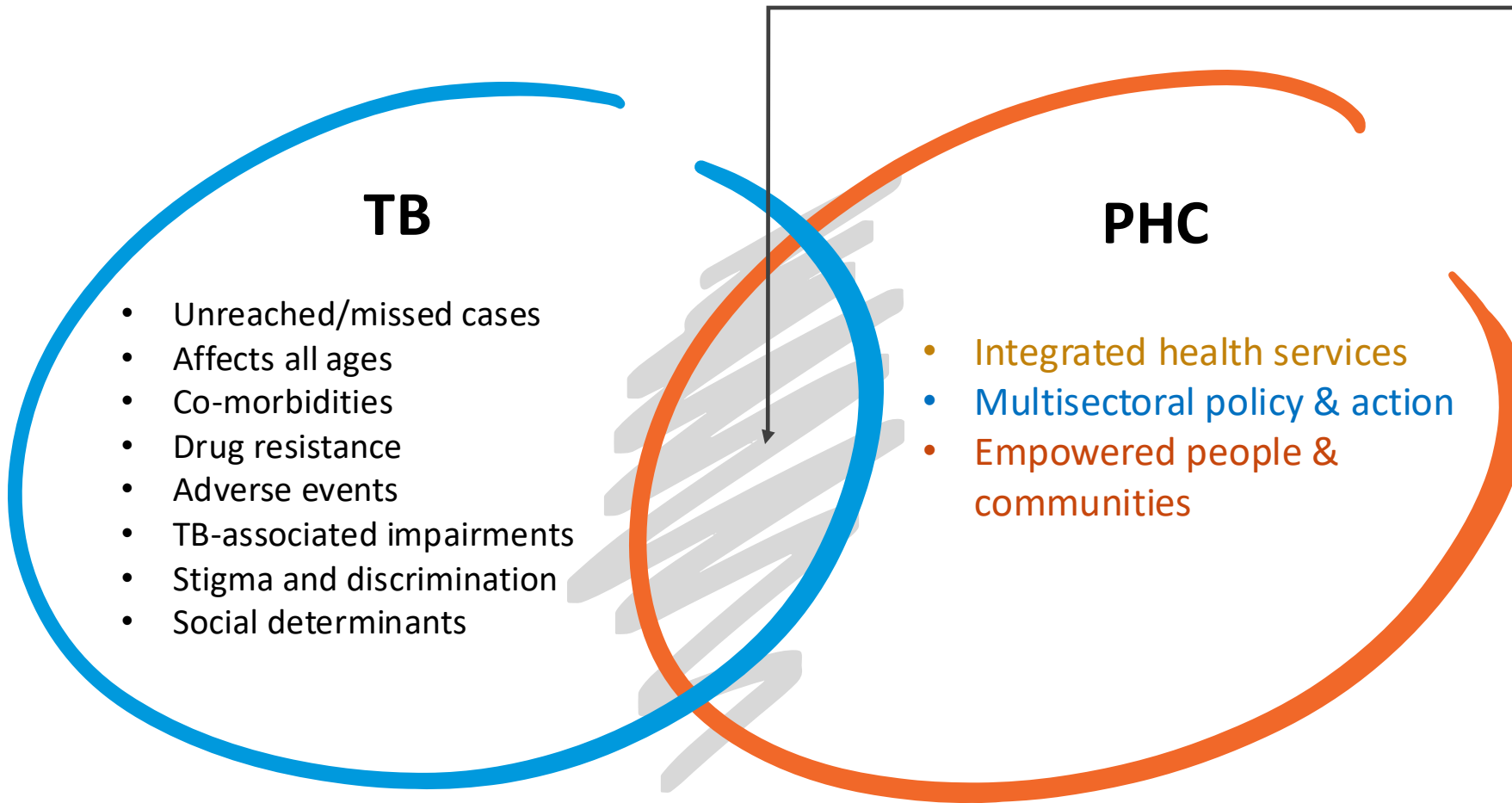
- TB remains a **leading infectious disease** globally, affecting millions annually and posing significant public health challenges
- Despite notable progress, global TB targets (UNHLM and End TB Strategy targets) are **off track**
- Significant **gaps** remain in global TB efforts largely owing to inadequate funding and fragmented financing systems



Reduction 2015–2020 was 12%; from 2010–2020, 22%



TB and PHC: why does this matter?



TB + PHC

Models of care that provide accessible, continuous, comprehensive, and coordinated first-contact, **people-centred services**

addressing broader **health determinants**, engaging **cross-sector stakeholders**

and **empowering** individuals, and communities to improve health and reduce disparities.

Scope of the policy brief



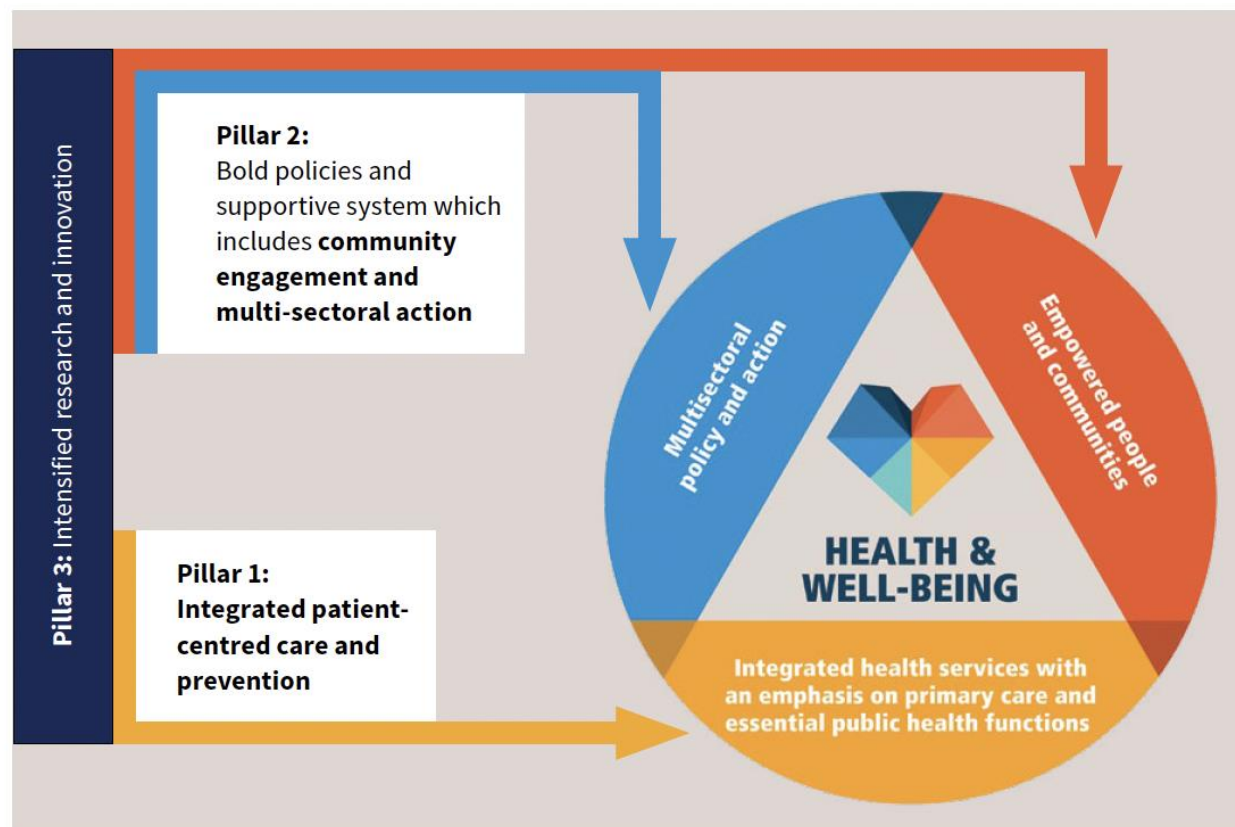
- **Policy brief objective:** To explore synergies between the TB response and PHC, identifying opportunities to strengthen both areas concurrently.



- **Target audience:** Aimed at policymakers, health managers, PHC and TB programme leaders, non-governmental organizations, civil society, affected communities, researchers, donors, and technical partners involved at various levels of the health system.

Synergies between the PHC approach and the TB response

Components of the PHC approach and the End TB strategy



PHC: primary health care; TB: tuberculosis.

Note: Pillar 3 of the End TB Strategy is linked to the PHC operational lever on primary health care-oriented research.



TB and PHC synergies: Integrated health services with an emphasis on primary care and essential public health functions

Integrated health services with an emphasis on primary care and essential public health functions

Brief definition: Meeting people's health needs through comprehensive promotive, protective, preventive, curative, rehabilitative, and palliative care throughout the life-course, strategically prioritizing key health care services aimed at individuals and families through primary care and the population, with essential public health functions as the central elements of integrated health services.

- **Integrated health services:** Provide continuous care across prevention, diagnosis, treatment, rehabilitation, and palliative care—within and beyond the health sector—throughout the life-course.
- **Benefits:** Better access, outcomes, efficiency, patient satisfaction, health worker morale, and cost-effectiveness.
- **Convergence point** Pillar 1 of the End TB Strategy calls for universal access to integrated, patient-centred TB care—addressing TB, comorbidities (e.g. HIV, diabetes), and broader health needs via PHC.
- Integration levels vary—from dedicated TB facilities to TB services embedded in primary care.
- **Essential public health functions:** Core actions to improve population health, guided by national essential health service packages covering the full spectrum of care (promotive to palliative).
- When we integrate TB into essential health service packages, we align resources, define coverage, and strengthen our entire health system.

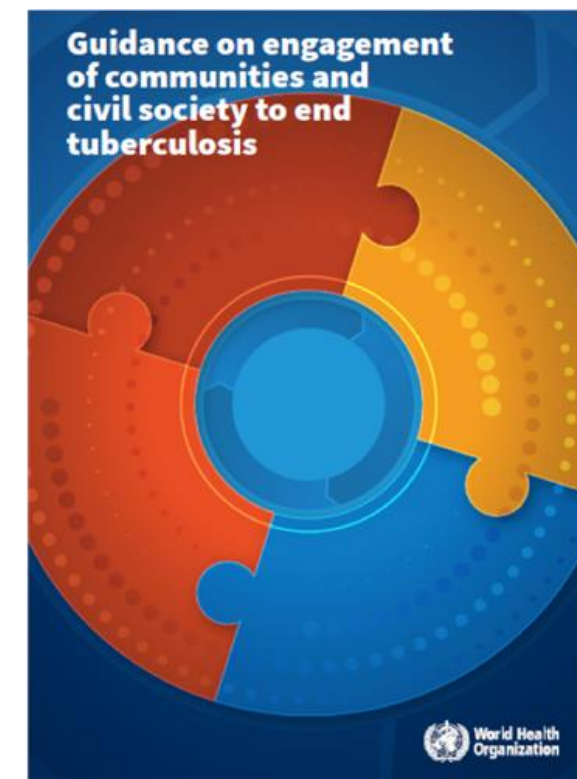


TB and PHC synergies: Empowered people and communities

Empowered people and communities

Brief definition: Empowering individuals, families, and communities to optimize their health, as advocates of policies that promote and protect the health and well-being, as co-developers of health and social services, and as self-carers and caregivers

- Communities are **at the heart of both the End TB Strategy and PHC**.
 - End TB Strategy: Communities & civil society are core to ending TB (Pillar 2).
 - UNHLM on TB: Calls for people-centred health services with full community engagement.
 - Communities often deliver TB treatment & care in many settings.
- **Convergence point:** PHC empowers communities as **equal partners** in prevention, care, and decision-making—bringing services closer to where people live and work.
- This is not just participation—it's partnership, ensuring the response is rooted in rights and people-centred approaches.



TB and PHC synergies: Multi-sectoral policy and action

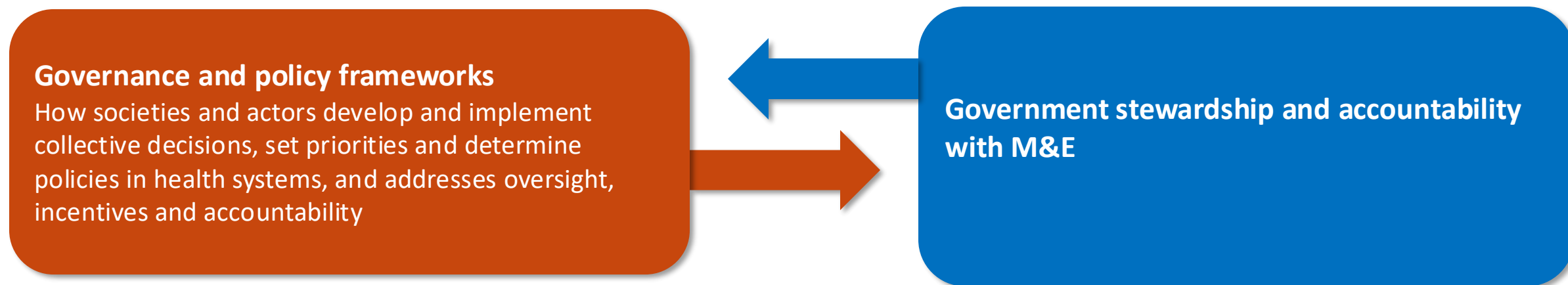
Multisectoral policy and action

Brief definition: Systematically addressing the broader determinants of health (including social, economic, and environmental factors, as well as individual characteristics and behaviour) through evidence-informed policies and actions across all sectors.

- Addressing TB means tackling health-related risk factors (HIV, undernutrition) and social determinants (poverty, poor housing, etc)
- This requires collaboration with sectors within and beyond health
- PHC approach fosters partnerships across sectors at both policy and local action levels, while TB programmes bring deep experience in multisectoral collaboration
- **Convergence point:** By mapping stakeholders and convening sectors, we can tailor actions to national and local contexts that benefit both the reorientation of health systems towards PHC and the TB response
- **WHO's MAF-TB Framework** helps us keep all actors accountable



Core strategic levers and End TB Strategy: Governance and policy frameworks



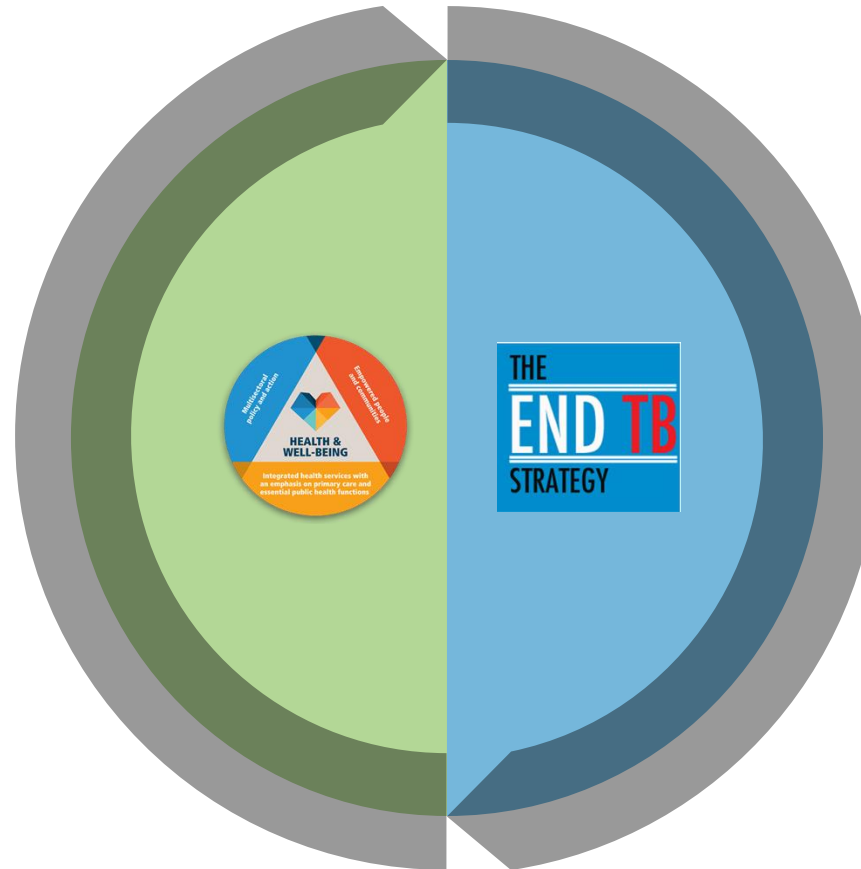
TB-PHC convergence:

- Political commitment from heads of state to local leaders is essential to reorient health systems toward PHC while achieving TB targets, integrating services, engaging communities and addressing the broader determinants of health through multisectoral policies.

Core strategic levers and End TB Strategy: Health financing

Health financing

- It is the role of health financing to mobilize sufficient resources to make PHC effective and, given the shortfall in public funding in so many settings, to seek to preserve access and equity, and protect patients from the (sometimes catastrophic) impacts of out-of-pocket payments.
- a crucial tool in reorienting health systems towards a PHC approach giving policy-makers the levers to achieve change.



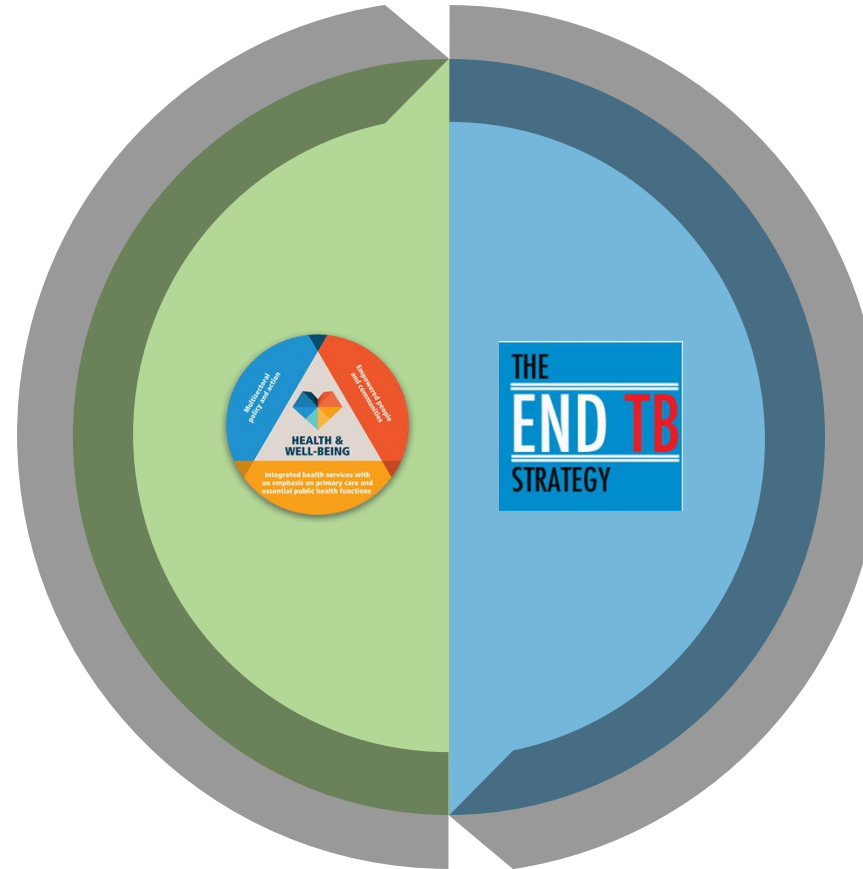
Mobilising resources for an ambitious agenda

TB-PHC convergence: **Analysing TB financing through a PHC lens can reveal opportunities to strengthen systems and reallocate funds for maximum impact.**

Operational levers and End TB Strategy: Health and care workforce

PHC workforce

- Expected to provide health promotion, prevention and public health services; deliver acute and chronic care; ensure continuity of care; and respond to patients' needs and expectations.
- Educating, attracting and retaining sufficient adequately-trained, motivated professionals is absolutely critical.
- Strategic planning, education, life-long training, recruitment, retention and distribution are essential.



Universal health coverage policy, & regulatory frameworks for case notification, vital registration, quality and rational use of medicines, & infection control

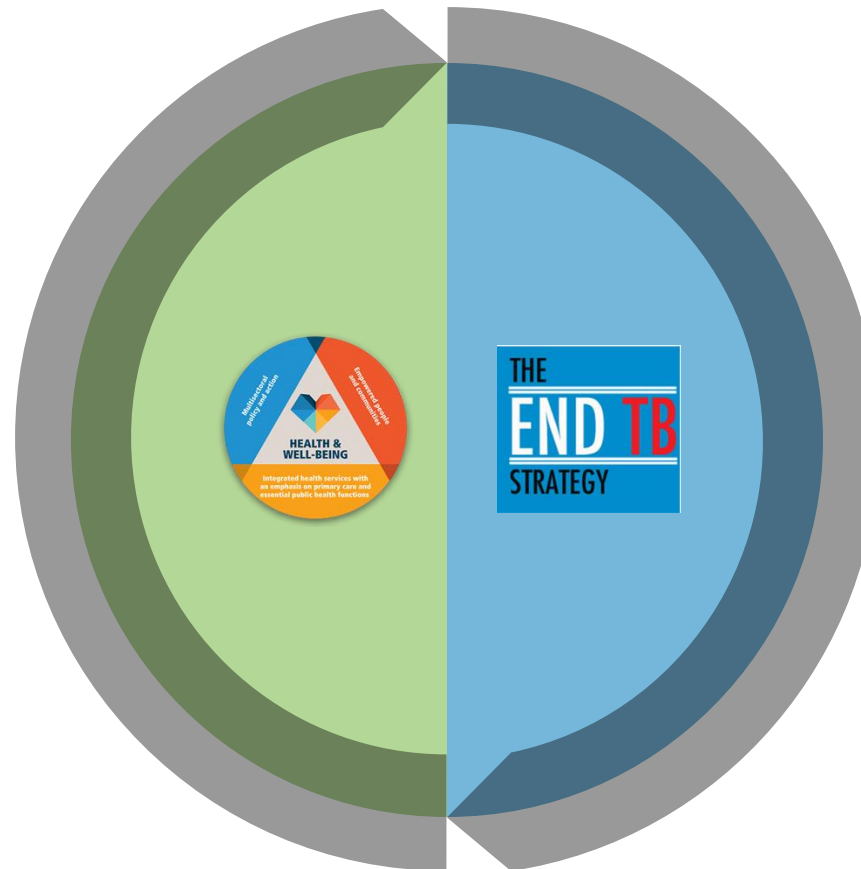
TB-PHC convergence: We need an adequate, motivated, well-distributed workforce capable of delivering both TB and PHC services, ensuring continuity and quality.

Operational levers and End TB Strategy: Models of care

Models of care

Define where and how services are delivered; multiple unaligned models can cause fragmentation. PHC-oriented models enable comprehensive, integrated, people-centred care over the life course.

Reorientation: complex, long-term, but improves quality, responsiveness, and efficiency. No single “correct” model—must fit national & local contexts.



Integrated patient-centred care and prevention

TB–PHC convergence: **Both agendas promote bringing services closer to communities, ensuring care is continuous, coordinated, and people-centred.**

Enablers and barriers to synergistic actions for the PHC approach and ending TB

Enablers of PHC reorientation of health systems for ending TB

1 The economic case for TB & PHC

2 Reaching the unreached

3 Environmentally friendly sustainable solution

4 Service continuation in emergency

5 TB as an entry point to health system reorientation



Barriers to PHC reorientation of health systems for ending TB



Inadequate political commitment



Overstretched primary care facilities



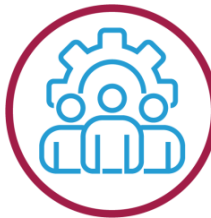
Health system inadequacies



Fragmented funding flow and donor dependency



Inadequate infrastructure



Inadequate health workforce



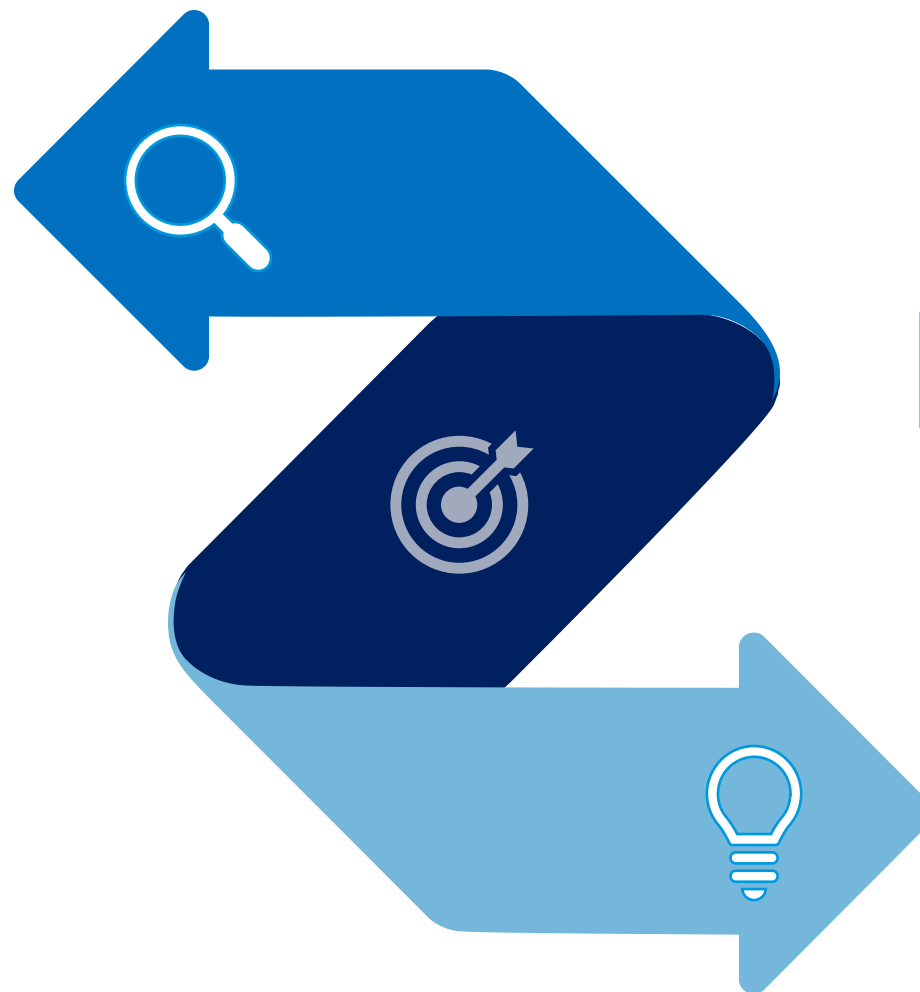
Inadequate data management & surveillance systems

Advancing towards PHC-oriented health systems for ending TB

Tailored actions in countries at an early and advanced stage of PHC policy environment to advance the TB agenda and PHC orientation of health system

Countries at an early stage

- Conduct a situation assessment
- Undertake joint planning
- Apply systems thinking and change management to guide sustainable reorientation toward PHC
- Engage all stakeholders; multisectoral actions require multiple stakeholders
- Advocate and be at the table
- Learn and share



Countries at an advanced stage

- Ensure sustainable funding
- Strengthen the health workforce, infrastructure and equipment
- Monitor implementation
- Using implementation research to adapt and share what works
- Learn and share

Monitoring progress – existing indicators



Integrated service delivery

- UHC service coverage index
- TB treatment coverage
- Number of PHC sites
- PHC sites with a WHO-recommended rapid diagnostic test (WRD)
- National electronic case-based database

Meaningful community engagement

- Referrals and new notifications
- Treatment success
- Community representation in national decision-making
- Level of committed funding for community engagement in the TB response at the national level

- Presence of a national multisectoral and multistakeholders accountability and review mechanism

Monitoring progress – additional indicators



Package of essential health services that includes TB

Presence of a package of essential health services in the country that includes TB



Proportion of primary care centre providing a package of TB services

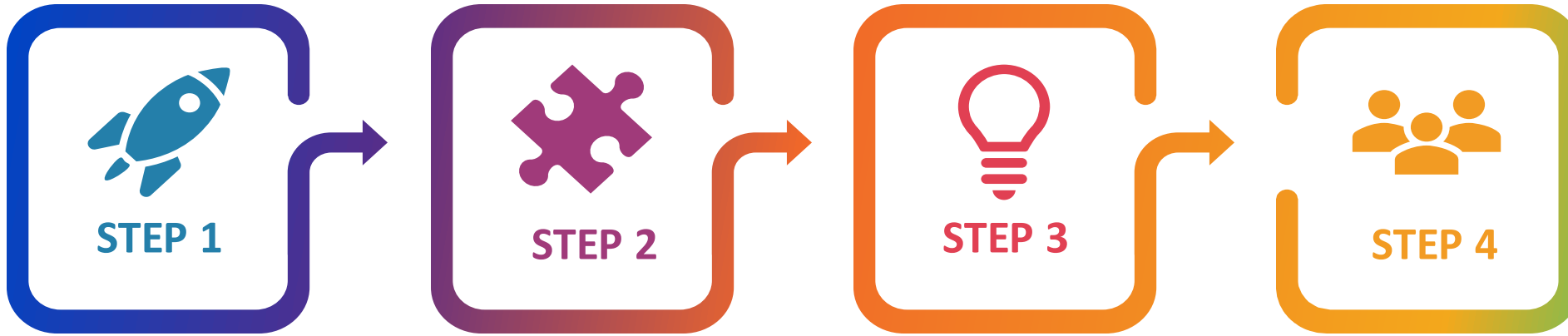
Proportion of PHC facilities in the country that provide a package of TB services out of the total number of primary care facilities



Health workforce

Presence of TB training in diagnosis, treatment and management for health workers at the primary care level

From policy to action: strengthening PHC, ending TB, advancing UHC



Working with countries and partners to translate policy ideas into tailored, actionable strategies that strengthen the PHC orientation of health systems and end TB

Documenting country experience (potential to inform future how-to)

Deep-dive into specific levers

Sharing country experience

Acknowledgments

Pedro Avedillo, Annabel Baddeley, Sepideh Bagheri, Anand Balachandran, Vineet Bhatia, Kenza Bennani, Tsolmon Boldoo, Annemieke Brands, Maria Regina Christian, Alarcos Cieza, Hannah Monica Dias, Ismael Endris, Dennis Falzon, Michel Gasana, Rabin Gautam, Lice Gonzalez Angulo, Christian Gunneberg, Patricia Hall-Eidson, Karina Halle, Laeeq Khawaja, Faraz Khalid, Setiawan Jati Laksono, Nomthandazo Lukele, Francis Mupeta, Fuad Mirzayev, Eunice Omesa, Challa Negeri Ruda, Mya Sapal Ngon, Andrew Seale, Lana Syed, Shamsuzzoha Babar Syed, Sabine Verkuijl, Kerri Viney, Martin van den Boom, Rajendra Yadav and Matteo Zignol.

WHO acknowledges the following people who led the drafting of specific themes within the document: Annabel Baddeley (WHO) on the framework for collaborative action on tuberculosis (TB) and comorbidities, and Sabine Verkuijl (WHO) and Anne Detjen (United Nations Children's Fund [UNICEF]) on TB and child and adolescent integrated health, and Lice Gonzalez Angulo on integrated approaches to TB and lung health.

WHO also gratefully acknowledges the contribution of the following people: Anne Detjen and Ruslan Malyuta (UNICEF), Kathy Fiekert (KNCV Tuberculosis Foundation), Robert Makombe (FHI 360), Mohammed Yassin (Global Fund to Fight AIDS, Tuberculosis and Malaria [Global Fund]), Guy Stallworthy (Bill & Melinda Gates Foundation), Immaculate Kathure, Stephen Macharia, Nellie Mukiri (Ministry of Health, Kenya), Kelly Safreed-Harmon (WHO consultant), Jacquie Oliwa (Kenya Medical Research Institute), Jaime Bayona Garcia (World Bank), Tiffany Tiara Pakasi (Ministry of Health, Indonesia), Tendai Nkomo, Sein Sein Thi, Joyce Sibanda (Ministry of Health, Eswatini), Moorine Sekadde (Ministry of Health, Uganda), Taposh K. Biswas and Gantungalag Ganbaatar (Ministry of Health, Mongolia), Edward Tisungane Mwenyenkulu (Ministry of Health, Malawi).

WHO thanks the peer reviewers for their invaluable feedback and input: Ghulam Qader Qader (WHO Civil Society Task Force on Tuberculosis), Md Khurshid Alam Hyder (TB and public health specialist) and Karina Kielman (Institute for Global Health & Development, Queen Margaret University).

Thank you!

Download the brief at: <https://www.who.int/publications/i/item/9789240111295>

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Regional and country perspectives: harnessing synergies and opportunities between TB and PHC



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Reflections from stakeholders: the PHC approach and TB: challenges and opportunities



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